

Name : Saroj Agarwal	Age/Gender : 63 y / F	Collected : 20/02/2026 21:16
Patient ID : 0010663186	Visit Type : IP	Received : 20/02/2026 23:05
Referred By : Dr. Darshit Shah	DOB : 27/08/1962	Reported : 21/02/2026 08:59
Accession : 2600041253	Location : 1T06N`1TR0601`1TB0605`1ONCOMED	

DEPARTMENT OF LABORATORY MEDICINE - BIOCHEMISTRY

	Test Name	Unit	Reference Range	Low	Normal	High
	Liver Profile					
	Bilirubin Total + Direct					
	Total Bilirubin (Serum, Diazo)	mg/dL	<= 1.2		0.28	
	Direct Bilirubin (Serum, Diazo)	mg/dL	<= 0.3		0.16	
	Indirect Bilirubin (Serum, Calculated)	mg/dL	<= 1.2		0.12	
	Alanine Aminotransferase [ALT/PT] (Serum, IFCC-Enzymatic with P5P)	U/L	<= 35		17.60	
	Aspartate Aminotransferase [AST/OT] (Serum, IFCC-Enzymatic with P5P)	U/L	<= 35		22.60	
	Gamma Glutamyltransferase [GGT] (Serum, Enzymatic IFCC)	U/L	< 40		26.90	
	Alkaline Phosphatase [ALP] (Serum, Enzymatic IFCC)	U/L	35-104		64.60	
	Proteins					
	Total Protein (Serum, Colorimetric - biuret)	g/dL	6.4-8.3	5.28		
	Albumin (Serum, Colorimetric - BCG)	g/dL	3.5-5.2	2.74		

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	Test Name	Unit	Reference Range	Low	Normal	High
	Globulin (Serum, Calculated)	g/dL	2-3.5		2.54	
	A / G ratio (Serum, Calculated)		1.0-2.5		1.08	

Total Bilirubin:

- Diseases or conditions which, through hemolytic processes, produce bilirubin faster than the liver can metabolize it, cause the levels of unconjugated (indirect) bilirubin to increase in the circulation.
- Liver immaturity and several other diseases in which the bilirubin conjugation mechanism is impaired cause similar elevations of circulating unconjugated bilirubin.
- Bile duct obstruction or damage to hepatocellular structure causes increases in the levels of both conjugated (direct) and unconjugated (indirect) bilirubin in the circulation.

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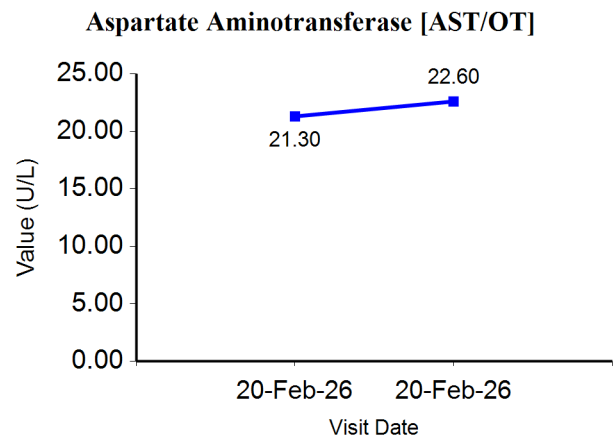
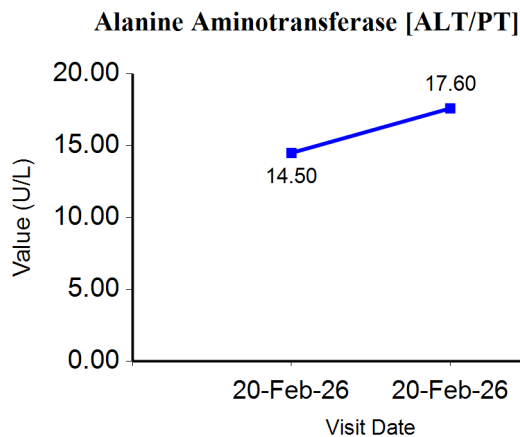
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Trend Analysis & Graph



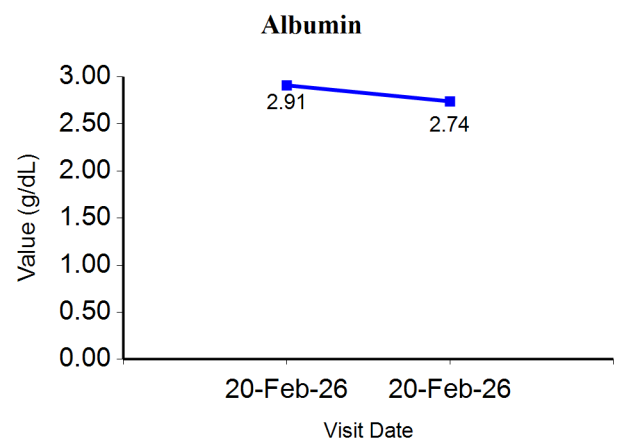
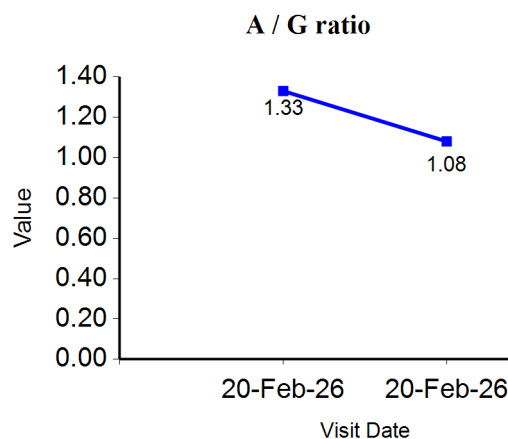
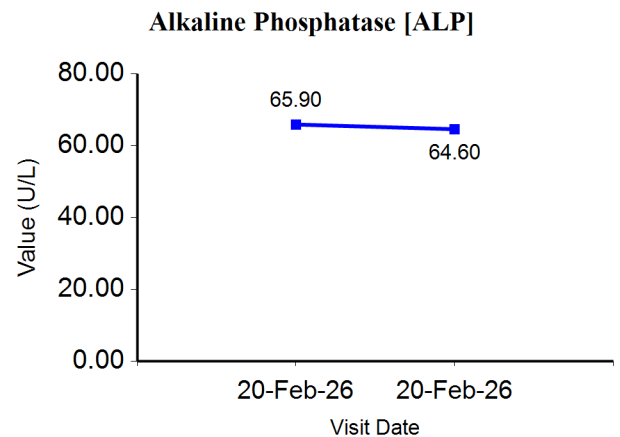
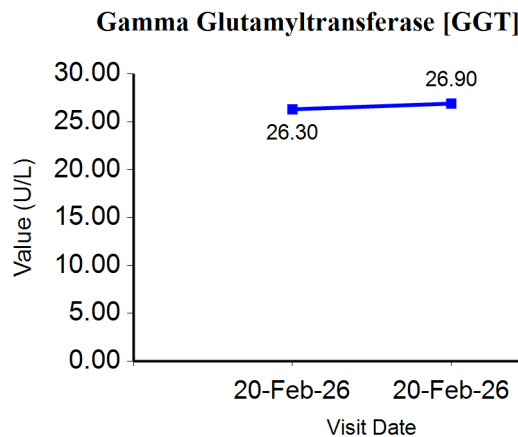
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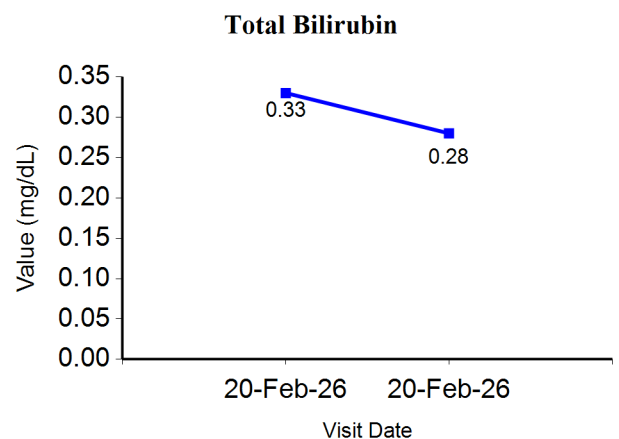
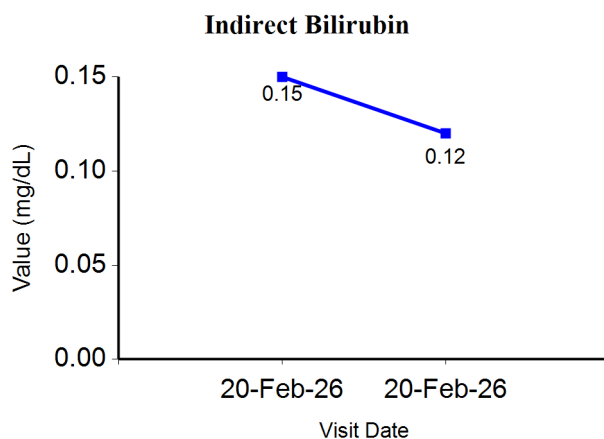
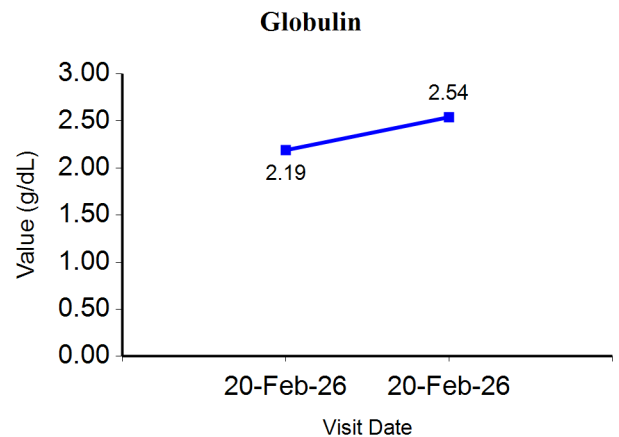
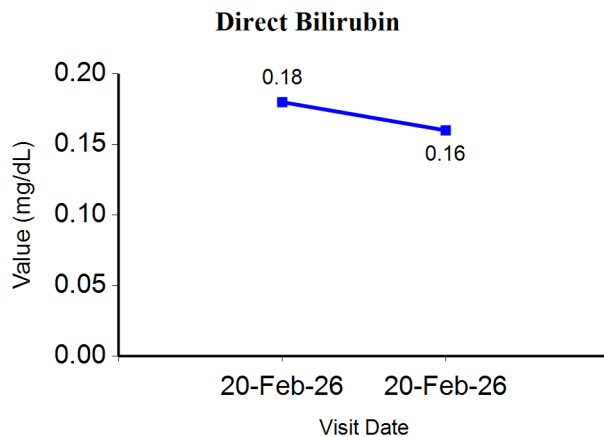


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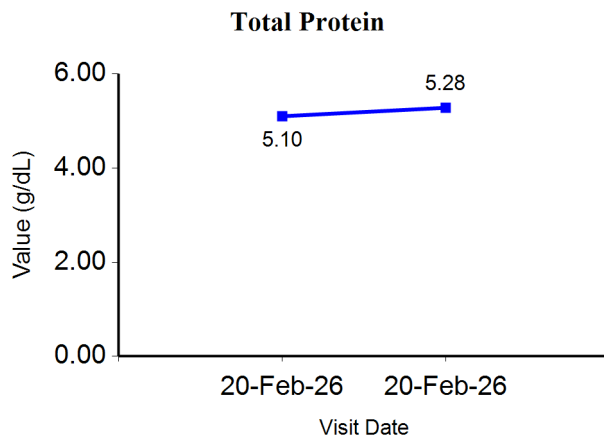
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∞ End of Report ∞

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